TEAM MEDVERSE / GATE 33

Commitment workspace integration

Experimental timing is visible, inspectable and causally replayed.

Integrity: PASS | 720/720 identical event states | 120/120 identical traces

Controller	Complete	Clinical	Wait steps	Invalid
Eager	14/32	28/30	0	0
Fair Bounded Wait	15/32	29/30	38	0
Medverse Commitment	15/32	29/30	38	0

Known Gate 32 cases; 32 correlated confirmation-history continuations, not clinical patients.

What changed

An operator interface now releases one event, exposes the actual timing disposition, shows each institution's contracts, and exports proposal-to-effect receipts.
The unchanged original clinical / Digital Twin application remains separately accessible.

What did not change

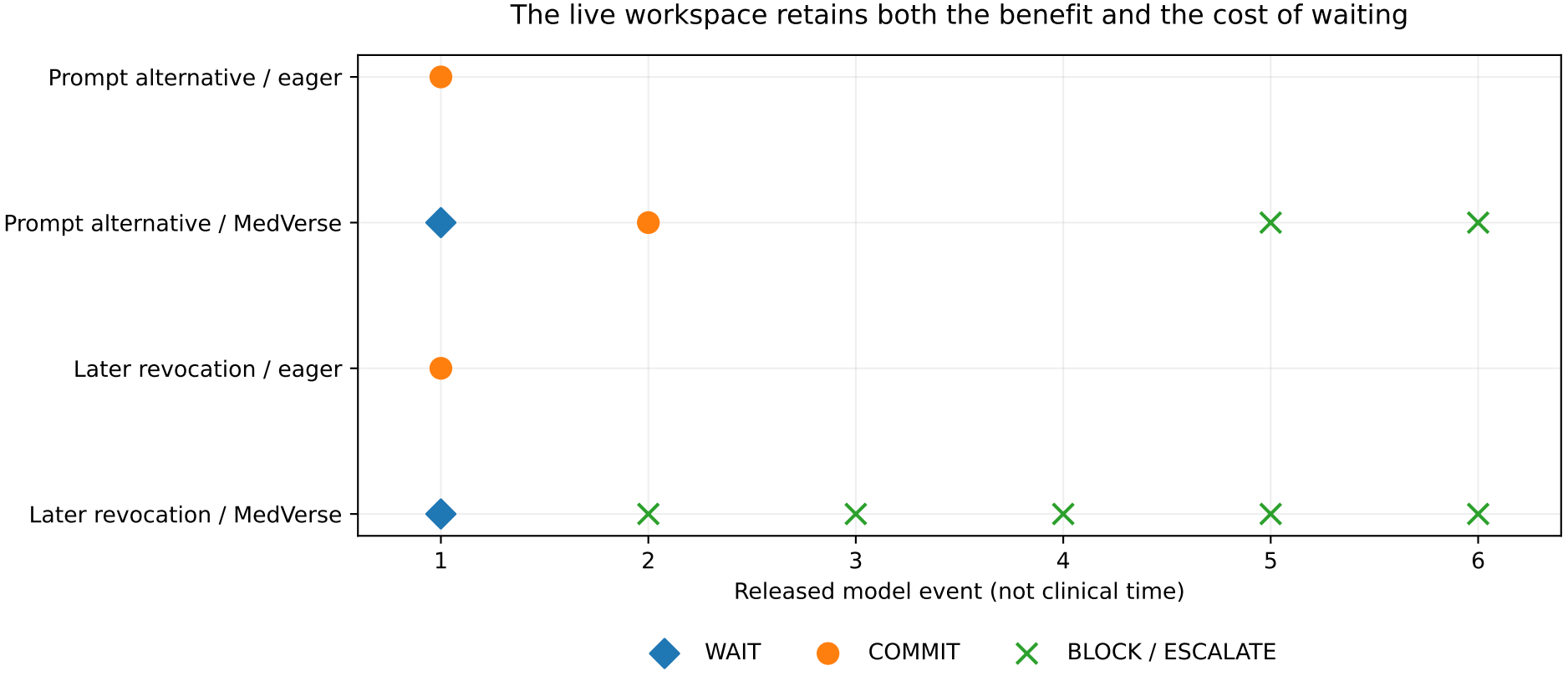
No controller, selector, deadline, observation, reference outcome or policy was retuned. Four gains and three losses versus eager commitment remain. The capable timing control still ties. C01/C02 were not retrospectively repaired.

Measured integration

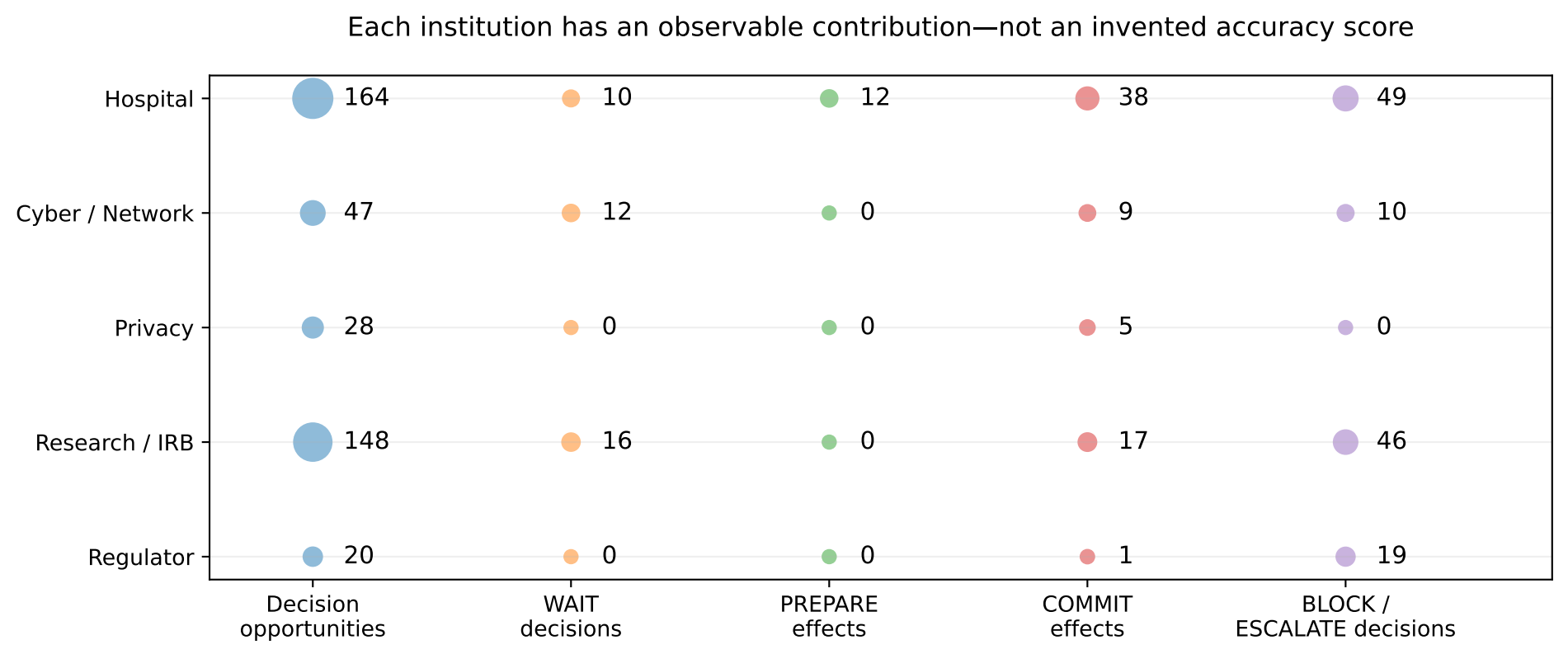
47/47 wrapper checks; 177/177 same-history comparisons;
72/72 precommit-change probes without new effects;
432/432 original clinical event outcomes preserved.

Evidence boundaries

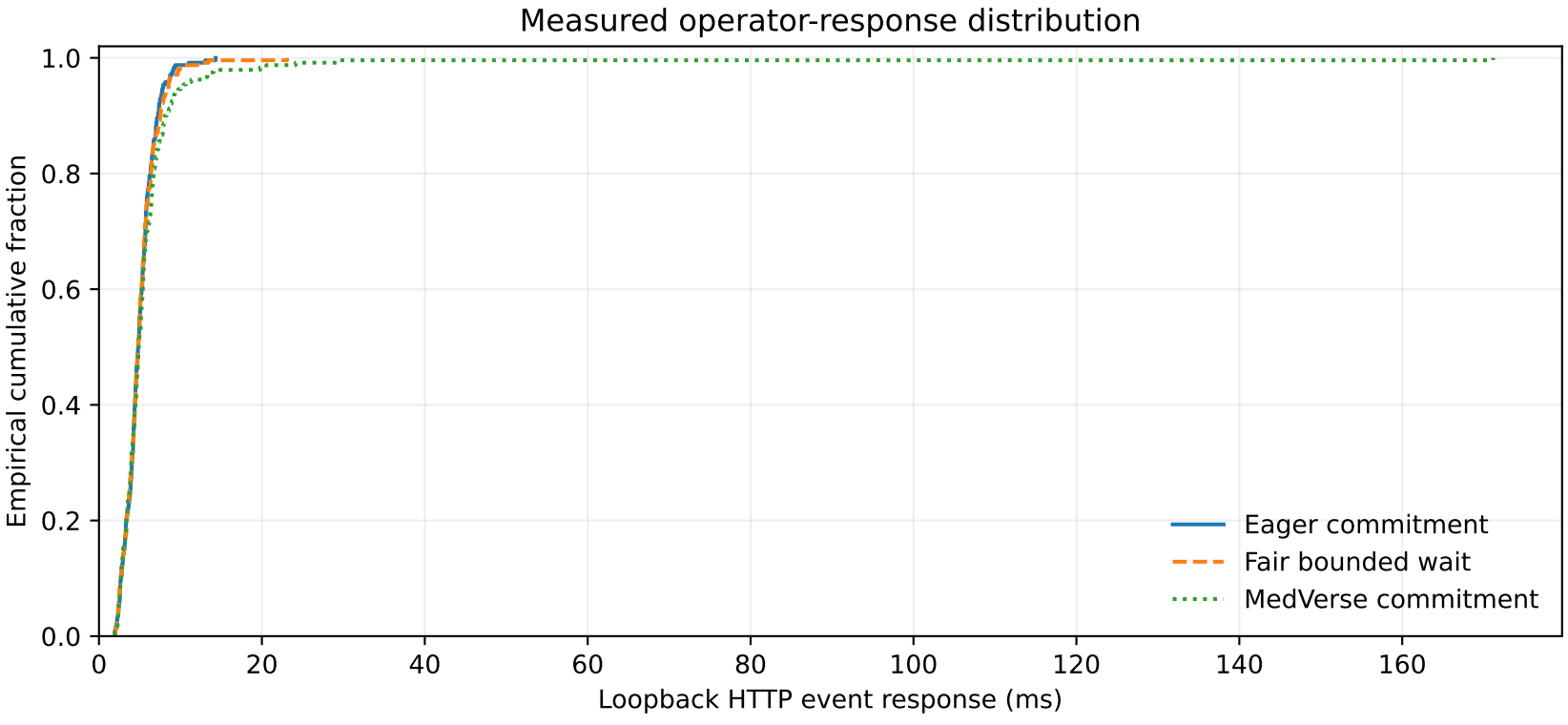
Modeled contracts and authority only. Clinical/legal review is pending. Role projections are operator views, not evidence of authenticated remote peer isolation. This is known-input integration regression, not new confirmation or clinical deployment validation.



P01 tracks clinical review; P04 tracks the research view. Actual HTTP receipts mark commitments.
No future proposals enter the controller; previously valid effects are not retroactively invalidated.



MedVerse commitment: 32 known continuations across 8 prefix families. Reconsideration is counted per event.
Counts are correlated opportunities/effects; a large count is not stronger coverage or better performance.



240 event calls per controller; one known-case replay, without randomized repetition.
Includes serialization and UI projection; excludes external network, LLM and human latency. No speed-superiority claim.